

SYNTHETIC DATA FOR TEACHING — NOT A REAL STUDY. Patients, investigators and institution are fictional.

This demonstration manuscript was written by AI (Claude Opus 5.5). In a real submission, the text must be written by the authors.

Response to the Editor and Reviewers

We thank the editor and the reviewers for their further review. We have addressed each remaining point below. In the marked revised manuscript, changes since the first revision are shown in blue and underlined (new text) and in red and struck through (deleted text), including the new column of Table 1; the Summary of Changes below gives the page of each change. The analysis of the weighted distribution of treatment periods was not prespecified and is labeled post hoc.

Summary of Changes

Page numbers refer to the marked revised manuscript. New text is shown in blue and underlined; deleted text in red and struck through.

Section	Part	Page(s)
Abstract	—	6
Materials and Methods	Study Design and Data	7
	Sources	
Materials and Methods	Statistical Analysis	10
Results	Patients	11
Results	Post Hoc Analyses	13
Discussion	—	14–16
Table 1	New column: SMD after	21

Section	Part	Page(s)
	overlap weighting; footnotes	
	revised	
Supplementary material	STROBE checklist provided as Supplementary File S2	—

Reviewer 1

Comment R1.1. *My earlier concerns have been addressed. However, the Conclusion still reads as if dual blockade caused* the improvement in EFS. For an observational study, please soften the causal language and state the main assumptions (no unmeasured confounding, positivity) in one sentence.**

Response: We agree that causal language is not appropriate for an observational study. We note that the Conclusion did not claim an EFS benefit: it stated that the data were insufficient to assess EFS or OS. Nevertheless, we have revised the wording so that no part of it can be read as causal. The Conclusion of the Abstract now begins “In this observational cohort, dual HER2 blockade was associated with a higher pCR rate,” and the concluding paragraph of the Discussion states that the data were insufficient to determine whether dual blockade is associated with longer EFS or OS. As requested, the concluding paragraph now states the main assumptions in one sentence: interpreting the association as an effect of the regimen assumes that no unmeasured factor influenced both treatment choice and pCR and that every patient could have received either regimen (positivity). We add that the positivity assumption was only partly met because the regimens were used in different years. We also replaced “the effect in patients for whom both regimens were realistic options” with “an association” in the Discussion.

Changes: Abstract (Conclusion); Discussion (third paragraph and concluding paragraph).

Reviewer 2

Comment R2.1. *Please provide the completed STROBE checklist as supplementary material, with page numbers.*

Response: The completed STROBE checklist was provided with the first revision as a separate file. It is now labeled Supplementary File S2, cited in the Methods, and listed in the Supplementary Material. The page number for each item refers to the clean revised manuscript and was updated for this revision.

Changes: Methods (Study Design and Data Sources); Supplementary Material (Supplementary File S2); STROBE checklist.

Comment R2.2. *Table 1 should show standardized mean differences before and after weighting (or matching), so readers can judge balance at a glance.*

Response: Table 1 now shows SMDs both before and after overlap weighting. After weighting, the SMD was 0.00 for every characteristic except the treatment period, for which it was 0.67. This finding is important, and we thank the reviewer for the request. Year of treatment start entered the prespecified propensity-score model as a continuous variable, so overlap weighting balanced the mean year exactly but not the distribution of treatment periods: in a post hoc analysis, 13.7% of the weighted dual-blockade group and 32.8% of the weighted single-blockade group started treatment in 2012–2015. We now report this in the Results, Discussion, and limitations. Two analyses address it. When year was modeled with a natural spline (post hoc analysis from the first revision; Supplementary Table S4), the SMD for treatment period after weighting fell to 0.17 and the OR for pCR was 2.00 (95% CI, 1.05–3.79), similar to the primary estimate (2.01 (95% CI, 1.12–3.62)). The prespecified analysis restricted to patients who started treatment in 2016–2019 gave an OR of 2.15 (95% CI, 0.92–5.01). We have kept the prespecified primary analysis and interpret it with this limitation.

Changes: Table 1 (new column and revised footnotes); Methods (Statistical Analysis, post hoc analyses); Results (Patients; Post Hoc Analyses); Discussion (third paragraph and limitations).

We hope that the revised manuscript is now suitable for publication in the Journal of Cancer Research and Practice.